

Q I've been breast-feeding for two weeks and my nipples are sore and cracked. What can I do?

Your breasts and nipples are not used to doing the job of breast-feeding so it is common for discomforts to occur, especially in the first few weeks.

It is important to address any physical discomforts early on, so that breast-feeding becomes a positive experience for you. If you dread any discomfort it causes, this won't be good for your well-being and it may affect how your baby nurses.

Sore and cracked nipples are a common discomfort, but should not be an ongoing problem once your baby is latching on correctly and nursing efficiently. Mostly because shorter feedings will mean your nipples get a break.

Problems such as cracked nipples can prevent your baby from nursing efficiently and lead to other discomforts. If milk isn't properly drained, it can cause blocked ducts and, in a worst-case scenario, mastitis. Both can be painful and make you feel sick so should be treated without delay.



Comfortable feeding Any physical discomforts of breast-feeding should be addressed early on.

BREAST-FEEDING: COMMON CONCERNS

Understanding the causes of breast-feeding discomforts can help prevent them from becoming worse. Take action as soon as possible to resolve any problems so that efficient feeding can be established.

PROBLEM	DESCRIPTION	CAUSES
Sore, cracked, and bleeding nipples	It's normal for your nipples to feel tender since your baby's sucking stretches them, but sore nipples that last beyond the first week can indicate a problem that needs addressing. If the nipples become cracked, you may experience sharp pain for a few seconds at the beginning of a feeding. Cracked nipples may also bleed, and your baby may swallow some blood. Streaks of blood may appear in your baby's stools or when he spits up. Although alarming, this isn't harmful for your baby.	If your baby isn't latched on correctly, he may suck on your nipple, rather than your breast tissue. This makes the nipple increasingly tender, and the pressure of the sucking can cause the nipple to crack, and sometimes bleed. Since your nipples are often moist when breast-feeding, it is hard for the cracks to heal. If nipples suddenly become red and sore after a period of being fine, you may have thrush, a fungal infection that can pass between you and your baby. You will both need medical care.
Blocked milk duct	A blocked duct can result in a lump with swelling and inflammation, and the area may be tender. The swelling may be alleviated slightly after breast-feeding. If bacteria reach the blocked duct, it can lead to mastitis (see below).	Milk ducts form a series of channels that carry milk to your nipple. A milk duct can become blocked if your breast isn't drained completely, often because your baby has a poor latch or has a weak suck. In the early days, when your breasts are producing a lot of milk, they may become engorged, leading to a blocked duct. A badly fitting bra can put pressure on a part of the breast and cause a blockage. If you have redness, swelling, and lumps, contact your doctor. Early treatment can help avoid further complications, such as mastitis.
Mastitis and breast abscess	If you have hardness, swelling, and redness, and your temperature is slightly raised, you may have mastitis. If the infection worsens, it can become very painful, your temperature may escalate, and you may have flu-like symptoms. Occasionally, pus forms in the area and an abscess develops, which feels like an extremely painful lump. Mastitis is most common in the early weeks of breast-feeding when your milk supply hasn't yet settled down.	Problems can stem from your baby having a poor latch or weak sucking, which leads to a blocked duct or an area where there is a build up of milk, referred to as milk stasis. If bacteria present on your skin travel to a blocked milk duct, it can cause a mastitis infection. If you are anemic, or very run down, your resistance may be lowered, making you more susceptible to infection.